Assessor

Take on candidates in your room, assess, and save the result

RMD Birmingham
Assessment night

Not here	Arrived	Collected	Being assessed	Assessed
On the list, not checked in	Checked in, waiting for runner	Runner has them, going to room	Assessor has taken them on	Result saved

WHO CAN ASSESS WHOM

- Senior Instructors do not do any assessments.
- **First sits:** assessed by the Assessor cohort.
- **Second sits:** need a different assessor from the first sit: RMD Senior Faculty, selected members of RMD Student Faculty (not all), or selected senior external assessors.
- **Third sits and later:** Senior Faculty or a named assessor. The hub tells you if you cannot take someone on.

ON THE NIGHT

1. Open the Phase 1 hub: **Assessment night**, then **Assessor (first sit)** or **Assessor (second sit)**. Third sits and later, and RAP candidates, are under **Retest-plus & RAP**.
2. Your room's list shows everyone expected, in slot order, with their state. Grey means **Not here**.
3. With check-in on, you can only pick someone who shows **Collected**. If they show **Arrived** and the runner is slow, tap **Collect now**, then **Yes, collect**. It is logged as collected by you.
4. Tap **Start assessment** (retests: **Take on**). They show **Being assessed** and nobody else can pick them. If someone got there first you see "[name] has taken them on".
5. Open the form. Mark **BLS + AED** and **First aid (recovery position)** separately as Pass or Not yet. For Not yet, tick the steps missed or add a comment. A Pass with a red step asks you why.
6. Tap **Save result**. They show **Assessed**. A referral at attempt 1 puts them in the retest pool as attempt 2.
7. Took on the wrong person? Tap **Put back** before you save.

If check-in is off: a Director decides each evening. If it is off you see everyone in your room and can pick anyone, as in earlier years. Start assessment and Save result work the same way.

COMMON FAIL REASONS

- Compression rate too fast or slow
- Unsafe delivery of shock
- Incorrect AED pad position
- Touching the patient during rhythm analysis
- Not checking breathing after the recovery position